CLINICAL DISCHARGE SUMMARY

SAMPLE / SYNTHETIC DOCUMENT FOR TESTING ONLY

Patient Name	Arun Kumar (Synthetic)
Date of Birth	12 March 1972
Gender	Male
Medical Record No.	TEST-47291
Date of Admission	18 August 2026
Date of Discharge	24 August 2026
Attending Physician	Dr. Meera Nair

1. Chief Complaint

The patient presented with a four-day history of fever, productive cough, fatigue, and shortness of breath on exertion. He reported reduced appetite but denied chest pain, hemoptysis, or recent travel.

2. History of Present Illness

Symptoms began approximately four days before admission. Fever was intermittent and associated with chills. Cough produced yellowish sputum. The patient noticed increasing breathlessness while walking upstairs. There was no known recent exposure to tuberculosis and no reported drug allergy.

3. Past Medical History

History of hypertension for approximately eight years, controlled with medication. History of type 2 diabetes mellitus for five years. No previous major cardiac surgery. No known chronic kidney disease.

4. Vital Signs on Admission

Temperature: 38.4 °C. Blood pressure: 148/88 mmHg. Heart rate: 104 beats/min. Respiratory rate: 22 breaths/min. Oxygen saturation: 92% on room air.

5. Physical Examination

Patient was alert and oriented but appeared mildly fatigued. Bilateral coarse crackles were heard over the lower lung fields, more prominent on the right. Heart sounds were regular without a new murmur. Abdomen was soft and non-tender. No peripheral edema was noted.

6. Investigations

Complete blood count showed WBC 13,800/µL with neutrophil predominance and hemoglobin 13.4 g/dL. C-reactive protein was elevated at 86 mg/L. Serum creatinine was 1.0 mg/dL. Chest radiograph demonstrated a right lower-lobe air-space opacity consistent with an infectious process. Blood cultures showed no growth after the standard observation period. Sputum culture was negative for a significant resistant pathogen.

7. Hospital Course

The patient was admitted for suspected community-acquired pneumonia with mild hypoxemia. He received supplemental oxygen initially, intravenous antibiotics, intravenous fluids, antipyretics, and glucose monitoring. His respiratory symptoms and fever gradually improved over the next several days. Oxygen saturation stabilized above 95% on room air before discharge. No intensive-care intervention was required. Blood glucose levels were monitored and his usual diabetes treatment was continued with adjustment during the acute illness.

8. Final Diagnoses

1. Community-acquired pneumonia, right lower lobe.
2. Mild acute hypoxemia, resolved.
3. Type 2 diabetes mellitus.
4. Essential hypertension.

9. Medications at Discharge

Amoxicillin/clavulanate 875/125 mg orally twice daily for 5 additional days.
Paracetamol 500 mg orally every 6 hours as needed for fever or discomfort.
Metformin 500 mg orally twice daily with meals.
Amlodipine 5 mg orally once daily.
Patient was instructed to complete the antibiotic course unless advised otherwise by the treating clinician.

10. Discharge Condition

Patient was afebrile, hemodynamically stable, tolerating oral intake, and able to ambulate without significant shortness of breath. Oxygen saturation was 96% on room air at the time of discharge.

11. Follow-Up and Instructions

Follow up with the primary-care physician within one week. Repeat clinical assessment and chest imaging may be considered if respiratory symptoms persist. Return urgently for worsening shortness of breath, persistent high fever, confusion, chest pain, bluish discoloration, or inability to maintain oral intake.

12. Clinical Notes / Evidence

The diagnosis was based on the combination of acute respiratory symptoms, fever, examination findings, inflammatory markers, and chest radiograph findings. There was no microbiological evidence of a resistant organism in the available cultures. The patient showed clinical improvement with treatment during hospitalization.